

Interview Transcript with Dr. Clara Hoffmann (Frankfurt, Germany) Pediatric Infectious Disease Specialist

Interviewer: Dr. Hoffmann, thank you for speaking with us. Could you tell us about your background and what drew you to pediatric infectious diseases? Dr. Hoffmann: I've always been fascinated by how resilient children are. I trained as a pediatrician in Frankfurt and specialized in infectious diseases about fifteen years ago. Before COVID, my work centered on measles, influenza, and childhood vaccination programs.

When the pandemic began, I never imagined our department would be thrust into the center of a global health emergency. Pediatric medicine is usually about prevention and reassurance. Suddenly, we were part of a fast-moving, uncertain storm.

Interviewer: Early in the pandemic, children were said to be largely spared. From your vantage point, was that accurate? Dr. Hoffmann: Partly. Children were indeed less likely to develop severe acute disease, especially in 2020. But 'spared' is misleading. We later discovered the post-infectious syndrome called MIS-C, which can be life-threatening.

More importantly, children suffered in indirect ways— isolation, missed schooling, fear. Pediatricians became mental-health counselors overnight. I spent as much time reassuring anxious parents as I did treating fevers.

Interviewer: How did your hospital prepare for pediatric COVID cases? Dr. Hoffmann: Frankfurt University Hospital designated an entire ward for pediatric COVID care. We re-trained our staff in adult-style respiratory support— something we'd rarely done before.

Initially, most children were mild cases admitted for observation because of parental anxiety. By late 2020, however, MIS-C began appearing: high fevers, rashes, shock, and cardiac involvement weeks after infection. We coordinated with cardiology and immunology units to design treatment pathways using IVIG and steroids.

Fortunately, outcomes were excellent when diagnosed early— but it required awareness. Education of pediatricians across Germany was key.

Interviewer: How did parents respond to the evolving science around COVID in children? Dr. Hoffmann: It was a roller coaster. In the beginning, parents were terrified their children would infect grandparents. Then came the opposite fear— that lockdowns were harming children more than the virus.

Our job was to offer perspective, not panic. I remember one mother sobbing because her 7-year-old hadn't played with friends in six months. We arranged a small, distanced outdoor play session. The child's laughter reminded us that healing isn't only biological— it's social.

Interviewer: What were the toughest decisions you faced regarding schools and lockdowns? Dr. Hoffmann: Those were deeply moral dilemmas. Pediatricians understood the necessity of

containment but also saw the damage of school closures – learning loss, loneliness, malnutrition for some.

I advised the state education board in Hesse and argued for keeping primary schools open with strict hygiene, rather than blanket closures. Children's risk was lower, and schools could implement masks, ventilation, and staggered schedules. Eventually, policy reflected that balance, though not without debate.

Science doesn't exist in a vacuum – it must engage with the realities of families.

Interviewer: What about vaccination for children? How was that handled in Germany? Dr.

Hoffmann: Initially, pediatric vaccination was approached cautiously. The European Medicines Agency approved vaccines for 12- to 15-year-olds mid-2021, and younger age groups later. Many parents hesitated, understandably, wanting long-term safety data.

We conducted numerous community seminars explaining mRNA technology in simple terms – It teaches the body, not changes it. That transparency helped. By early 2022, vaccination uptake among adolescents in our region exceeded 70%.

Vaccination also brought relief to families with vulnerable members – it restored intergenerational contact. Grandparents could hug grandchildren again. Those were emotional milestones.

Interviewer: Did your department conduct or participate in any research during the pandemic?

Dr. Hoffmann: Yes, several studies. We contributed to a national pediatric COVID registry tracking outcomes and to an antibody serosurvey among schoolchildren. Results showed that infection rates in children were often higher than detected by testing – many had mild or asymptomatic infections.

We also studied long-COVID symptoms in adolescents – fatigue, concentration problems, headaches. The challenge was distinguishing post-viral effects from psychological stress. It taught us that data must always be interpreted with empathy.

Interviewer: How did you manage communication with children themselves? Dr. Hoffmann: That was one of the most rewarding aspects. We designed child-friendly explanations: colorful posters about handwashing and short animations showing virus vs. immune system battles.

When vaccinating kids, we turned it into a game – stickers, applause, and a bravery bell they could ring afterward. Small rituals reduce fear. Children taught us that humor is powerful medicine.

Interviewer: Were there moments that moved you personally? Dr. Hoffmann: Many. One teenage boy with MIS-C was on ECMO for three weeks. When he finally recovered, the nurses lined the corridor and clapped as he left. His mother whispered, 'You gave me my child back.' I cried behind my mask.

Another moment was seeing pediatric staff volunteer for adult COVID wards when shortages peaked. They said, Patients are patients. That solidarity across specialties was inspiring.

Interviewer: What do you think Germany's pediatric system learned from this crisis? Dr.

Hoffmann: Several lessons. 1. Preparedness must include children. Early pandemic plans barely mentioned pediatrics. 2. Mental health is part of infection control. Isolation can wound as deeply as disease. 3. Communication determines compliance. When we explain honestly, families cooperate.

We also learned to integrate data systems faster. Our digital registry now connects pediatric ICUs nationwide something we lacked before.

Interviewer: How did you support your team emotionally? Dr. Hoffmann: We made space for reflection. Weekly coffee rounds where hierarchy disappeared nurses, residents, consultants sat together and shared frustrations. Sometimes we talked about science; other times, about fear or exhaustion.

We also partnered with child psychologists who helped us process grief after pediatric deaths abroad, even though we had few locally. Grief travels through empathy. Taking care of caregivers became essential.

Interviewer: Germany often emphasized evidence-based decision-making. Did that help in pediatrics? Dr. Hoffmann: Yes, mostly. The Robert Koch Institute and the German Society for Pediatrics issued timely, data-driven updates. But sometimes, evidence lagged behind public anxiety. For example, debates about mask safety in young children ignored existing respiratory data.

Evidence is only as useful as its communication. I learned that numbers persuade scientists, but stories persuade society. As pediatricians, we must tell both.

Interviewer: If you could change one aspect of Germany's pandemic strategy regarding children, what would it be? Dr. Hoffmann: I would have invested earlier in ventilation and air-quality infrastructure in schools. That would have reduced transmission and avoided so many closures. It's a long-term public-health investment, not just a pandemic fix.

Also, more unified messaging. Mixed signals federal vs. state vs. media confused parents. Consistency builds confidence.

Interviewer: And on a personal level, how has this experience changed you as a doctor? Dr.

Hoffmann: It made me slower in a good way. I listen more deeply now. Before, I sometimes rushed to reassure; now, I pause, acknowledge fear, and then explain.

It also reminded me that pediatrics is not just about small bodies it's about whole families. Healing a child often means healing parental anxiety.

Most of all, I realized how connected we all are. A virus that began thousands of kilometers

away reshaped every bedtime story in Germany. Medicine truly has no borders.

Interviewer: Any final reflections for future pediatricians? Dr. Hoffmann: Yes. Never underestimate children's wisdom. During a vaccine clinic, a 10-year-old told me, "You doctors look tired, but you're still smiling. That's the spirit we must keep—resilience through kindness."

To future doctors: study hard, but also play. Science saves lives, but laughter sustains them.

Interviewer: Beautifully said, Dr. Hoffmann. Thank you for your time and for your service.

Dr. Hoffmann: Thank you. It's been an honor to care for the smallest voices in a very big story.

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